Machnet
Medical Robotics

MN-RG-2026-01 · Rev C



MACHNET MEDICAL ROBOTICS · CORONARY PLATFORM

Reimbursement Guide 2026

Coding and payment for robotic-assisted diagnostic catheterization and PCI under Medicare and commercial contracts. United States.

FOR HOSPITAL LEADERSHIP FINANCE · CATH LAB · VALUE ANALYSIS

Robotic cases are paid the same way as manual cases.

One case rate per procedure. The rate is set by the procedure performed, not by the tools used to perform it.

Medicare and commercial payers pay a hospital **one case rate** for each coronary procedure. It covers the stents, balloons, wires, imaging and the robotic cassette used in the case. No additional code is required to bill a robotic case, and no additional code pays for one. The question for hospital leadership is how much of each case payment the consumables use, and how much remains.



The case is driven from the control console; the codes and the payment are unchanged.

$3,312

Medicare payment, diagnostic catheterization (C-APC 5191, CY2026)[2]

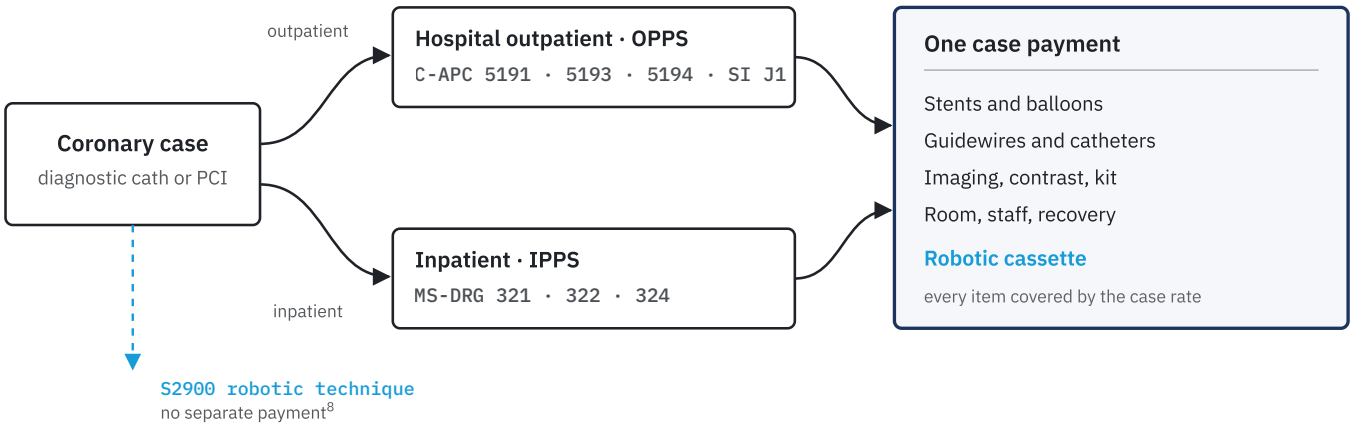
$11,794

Medicare payment, single-vessel stent PCI (C-APC 5193, CY2026)[2]

1.33–1.54×

National commercial carriers relative to Medicare, single-vessel stent PCI, posted hospital contracts[6]

How a robotic coronary case becomes a payment



The setting decides which payment system applies. The case rate covers every item used, the cassette included. Robotic assistance does not add a billable line.

TWO CASE TYPES, TWO CASSETTES, TWO PAYMENTS

CASE	CODES	CASSETTE	MEDICARE CY2026[2]	COMMERCIAL MEDIAN[6]
Diagnostic catheterization	93458 · C-APC 5191	$650	$3,312	$10,037
PCI, single-vessel stent	92928 · C-APC 5193	$1,500	$11,794	$18,356

WHAT STATUS J1 MEANS FOR THE CASSETTE

J1 marks a *comprehensive* APC: Medicare makes a single payment for the primary service and packages every adjunctive item on the claim into it. A device without its own pass-through code is reported with revenue code 0272 or 0278 and its charges roll into that payment. Adding the cassette does not add a claim line that pays and does not put the payment at risk. It changes the cost of the case, not the payment.

2026 HOSPITAL OUTPATIENT CODING AND PAYMENT

The codes that pay the case

CPT[1] codes most often reported for robotic-assisted coronary work, with CY2026 Medicare national rates and the commercial median from posted hospital contracts. Medicare rates are unadjusted national averages.[2]

CPT[1]	DESCRIPTION	STATUS[4]	APC	MEDICARE[2]	COMMERCIAL MEDIAN[6]
93458	Left heart catheterization with coronary angiography	J1	**5191**	$3,312	$10,037
92920	Coronary angioplasty, single vessel	J1	**5192**	$5,815	$12,036
92928	Coronary stent placement, single vessel	J1	**5193**	$11,794	$18,356
92933	Coronary atherectomy with stent, single vessel	J1	**5194**	$18,729	$25,615
92941	PCI for acute myocardial infarction	C	—	Inpatient only[5]	$14,563

2026 INPATIENT CODING AND PAYMENT

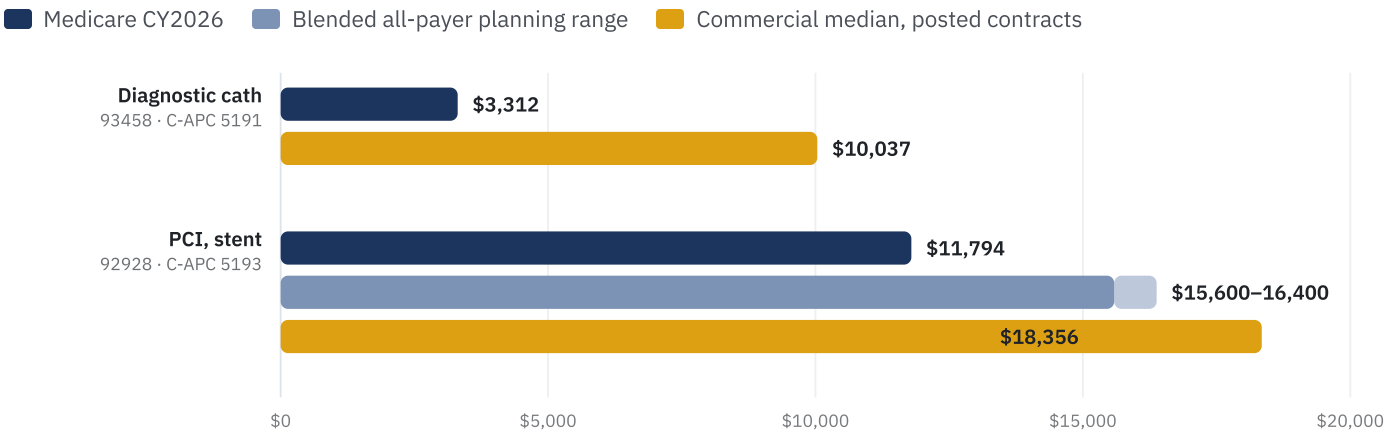
MS-DRG	DESCRIPTION	MEDICARE[3]	COMMERCIAL[6]
322	Percutaneous cardiovascular procedure with intraluminal device, without MCC	$13,109	$28,435
321	Same, with MCC or four or more arteries or devices	$20,051	$42,005
324	Coronary intravascular lithotripsy with intraluminal device, without MCC	$23,571	$46,666

Inpatient status is a medical-necessity determination under the two-midnight rule, not a billing choice. Physician services are paid separately and are outside this guide.

WHAT COMMERCIAL PAYERS PAY FOR THE SAME CASE

Commercial contracts pay more for the same case

Commercial contracts are negotiated on the same architecture: one case rate per procedure, packaged. For a single-vessel stent PCI, posted hospital contracts place the national carriers at **1.33 to 1.54 times** the Medicare rate (Humana 1.33, Aetna 1.44, UnitedHealthcare and Anthem 1.51, Cigna 1.54). The gap is widest on diagnostic catheterization.[6]

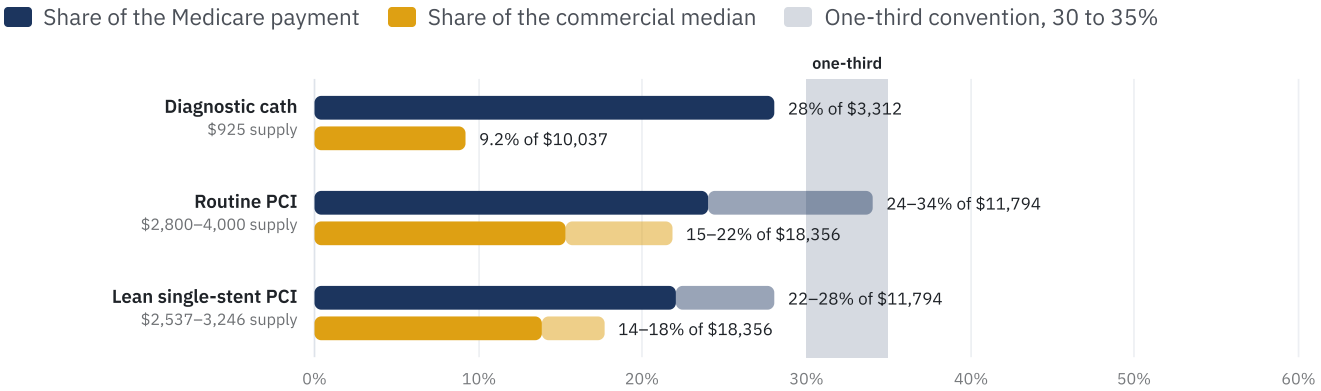


Commercial medians are from posted hospital contracts; the 92928 median reflects 21,426 commercial rates at 1,943 hospitals with a 25th to 90th percentile spread of about $13,200 to $36,400, so any single account should be planned on its own posted rates. The blended range weights Medicare, Medicaid and commercial volume at a typical mix.[6]

SUPPLY COST INSIDE THE PAYMENT

Where the room is

Cath-lab finance commonly holds total consumables to about a third of the case payment. Measured at one program's own purchase prices, every case type with its cassette included sits at or under that line on Medicare alone, and well under it on commercial.[7]



Supply figures include the cassette: $650 on the diagnostic case, $1,500 on the PCI cases. Lighter segments span the low to high build.[7]

DIAGNOSTIC CATH · 93458 $650 cassette

	MEDICARE[A]	COMMERCIAL
Case payment	$3,081–3,102	$10,037
Direct case cost, measured	– $1,650	– $1,650
Robotic cassette	– $650	– $650
Remaining per case	**≈ $800**	**≈ $7,700**

[a] One account's net collections, 93% of the APC rate. The diagnostic tier clears the cassette on Medicare alone.[7]

STENT PCI · 92928 $1,500 cassette

	MEDICARE	COMMERCIAL
Case payment	$11,794	$18,356
Consumables, measured	– $1,300–2,500	– $1,300–2,500
Robotic cassette	– $1,500	– $1,500
Remaining per case	**$7,800–9,000**	**$14,400–15,600**

Before room, staff and recovery cost, which the case rate also covers. One program's measured builds.[7]

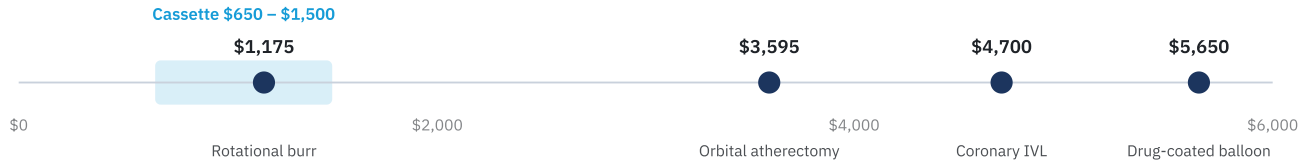
PRECEDENT

Coronary intravascular lithotripsy entered the market as a packaged adjunct disposable. Its stent cases now group to their own MS-DRGs (323 to 325), paying $10,500 to $12,700 more than the standard PCI DRGs. The payment system recognizes devices with an evidence case; it starts them inside the existing case rate.[3]



The robotic drive: a single-use cassette on a fixed case rate.

What the lab already pays for single-use devices in a PCI



One program's acquisition prices, August 2026: three of the four cost more than the PCI cassette, inside the same case rate.[7]

1 CPT® is a registered trademark of the American Medical Association. CPT © 2025 American Medical Association. All rights reserved. Fee schedules and relative values are not assigned by the AMA and are not part of CPT.

2 CMS CY2026 Hospital Outpatient Prospective Payment System, Addendum B as updated October 2026. National unadjusted payment rates; actual payment is adjusted by the hospital's wage index and is subject to beneficiary cost sharing.

3 CMS FY2027 Inpatient Prospective Payment System final rule as corrected (CMS-1849-CN3, in effect October 1, 2026), Table 5: relative weight (DRG 322 1.7744; 321 2.7140; 324 3.1905) times the unadjusted standardized amount of $7,387.95 (operating $6,848.00 and capital $539.95). Actual payment varies with wage index, DSH, IME and outliers. MCC: major complication or comorbidity.

4 Status indicator J1: comprehensive APC. A single payment is made for the primary service and packages adjunctive services, supplies and devices reported on the same claim.

5 Status indicator C: inpatient-only procedure. 92941 is paid under IPPS and is not payable in the hospital outpatient setting.

6 Commercial medians are hospital facility rates negotiated by commercial carriers for their commercial plans; the carriers' Medicare Advantage, Medicaid managed-care and exchange plans are excluded. Derived from hospital price-transparency machine-readable files as served in Alpha VZN's hospital-rate database on November 5, 2026 (facility rates, exact duplicates once, minimum-maximum stand-ins excluded): 93458 27,815 rates at 2,260 hospitals; 92920 22,815 at 2,012; 92928 21,426 at 1,943; 92933 16,984 at 1,664; 92941 5,601 at 330; MS-DRG 322 16,827 at 2,257; 321 16,734 at 2,248; 324 15,586 at 2,164. The blended range weights the Medicare and commercial case rates at a planning payer mix (51% Medicare, 13% Medicaid at 0.85 of Medicare, 29% commercial, 3% self-pay, 4% other), a 52% outpatient share and the 2024 Medicare discharge mix across MS-DRGs 321 to 325; its lower end treats a third of the commercial share as Medicare Advantage. Commercial payment varies by hospital and plan.

7 Supply builds and device prices are one cardiac program's GPO line prices, inventory-verified August 2026 (66 line items), and its net collections and cost accounting for pure diagnostic catheterizations over one year. Other programs will differ.

8 HCPCS S2900, surgical techniques requiring use of robotic surgical system, is a temporary national code not recognized for Medicare payment; where accepted by commercial payers it is packaged and carries no separate allowance.

REIMBURSEMENT QUESTIONS

Machnet reimbursement support

[email address · telephone · to be supplied]

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